



CLINICAL PERFORMANCE LAB
The Cheat Sheet Library

VOL III OF IV

DDx Ranking & Key Findings

How iHuman scores your differential — and
why most students rank wrong

The Assessment section is where clinical reasoning shows. Faculty
scrutinize this part — and the platform's auto-scoring is strict.

FREE RESOURCE

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Vol III of IV

DDx Ranking & Key Findings

How iHuman scores your differential — and why most students rank wrong

Here's the thing about iHuman's differential diagnosis scoring:

It's not just asking you to list possible diagnoses. It's testing whether you can rank them correctly — most likely first, must-not-miss prominently positioned, and ruled-out conditions explicitly addressed.

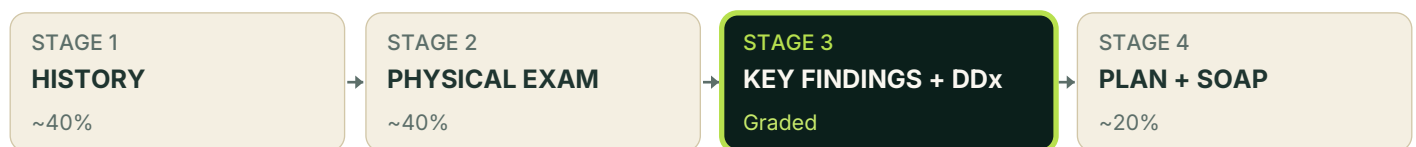
Get the ranking wrong and the score drops even when you have the right diagnosis on the list.

Faculty notice this. And on top of the auto-scoring, the problem statement is faculty-scored — they're checking whether your 2-3 sentence summary captures the case clinically and uses the right level of medical language.

This sheet covers how to think about DDx ranking, how to write a strong problem statement, and how to select key findings without padding the list.

These are skills, not memorization. Apply them across every case.

THE IHUMAN CASE FLOW · You are here



The Three Buckets of DDx

Every differential diagnosis on iHuman falls into one of three categories. Your ranking should reflect this:

1. Most-Likely Diagnoses — supported by history + PE + risk factors

Examples: GAS pharyngitis with Centor 4/4; Stage 2 HTN with target organ damage

2. Must-Not-Miss Diagnoses — less likely but high consequence if missed

Examples: Meningitis on any headache case; OSA on any HTN/snoring case; appendicitis on RLQ pain

3. Ruled-Out (or Less Likely) Diagnoses — considered and excluded based on evidence

Examples: Hypothyroidism (ruled out by normal TSH); secondary HTN (ruled out by normal renal labs)

The structure of your DDx list should mirror these buckets — most likely first, must-not-miss prominently, ruled-out near the bottom with explicit ruling-out rationale.

How iHuman Auto-Scores DDx

Selection matters. Each DDx you select scores positive if it's a valid consideration, negative if it's harmful or wildly off-track.

Ranking matters. The platform weights how you order the list. The final/primary diagnosis should be ranked #1.

Must-not-miss matters separately. A case with a must-not-miss diagnosis (e.g., cluster headache on a migraine case, OSA on an HTN case) scores additional points only if the must-not-miss is explicitly included on your list.

Verbatim names matter. iHuman uses a fixed dictionary of diagnosis names. If your platform expects "Acute viral gastroenteritis" and you select "Stomach bug," you get zero. Always pick the exact platform diagnosis name.

Writing the Problem Statement

PROBLEM STATEMENT · 3-sentence structure

Faculty-scored. Clinical language only. Brevity wins.

Who + CC + Duration

S1 H.H. is a 57-year-old Hispanic male who presents with elevated blood pressure noted at a community health fair, with no previous follow-up.

Key positives + Risk factors

S2 Examination reveals BP 172/94 (L) and 178/98 (R), AV nicking on fundoscopy, and laterally displaced PMI; risk factors include 35-pack-year smoking, family history of premature CVD, and high-sodium diet.

Diagnostic impression + Must-not-miss

S3 Findings are consistent with Stage 2 essential hypertension with target organ damage; OSA is a must-not-miss given snoring, obesity, and HTN.

What Faculty Look For

Clinical language — no lay terms, no patient quotes

Inclusion of both positives and risk factors — not just symptoms

Naming both the primary AND must-not-miss diagnoses

Appropriate medical brevity — 3 sentences, not 6

Centor Criteria — Sore Throat Cases

CENTOR CRITERIA · GAS pharyngitis probability

One point per criterion present (plus age modifier)

☐ Tonsillar exudate	1 point
☐ Tender anterior cervical lymphadenopathy	1 point
☐ Fever (>38°C / 100.4°F)	1 point
☐ Absence of cough	1 point
☐ Age 3–14 (modified Centor)	+1
☐ Age 15–44	0
☐ Age ≥45	–1

INTERPRETATION

- 0–1** Low risk — no testing, no treatment
- 2–3** Test (rapid strep or culture). Treat if positive.
- 4–5** High risk — empiric antibiotics may be appropriate

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Selecting Key Findings — The Goldilocks Problem

iHuman gives you a list of 12–20 findings on most cases. You're asked to select the ones that are present and clinically relevant.

Too many (selecting everything): Lowers score — flags poor clinical judgment.

Too few (selecting only obvious ones): Lowers score — misses pertinent positives.

Goldilocks (5–10 selections on a typical case): Strongest signal of clinical reasoning.

Select if it's:

A direct symptom or sign of the working diagnosis

A risk factor that strengthens the diagnosis

A pertinent negative that rules out a serious alternative

Don't select if it's:

- Normal/baseline (unless explicitly a pertinent negative for must-not-miss)
- Tangentially related to another finding already selected
- An incidental finding with no clinical bearing on the case

Must-Not-Miss by Chief Complaint

Every case has at least one must-not-miss diagnosis. Faculty award points for including it in the DDx, even when you correctly identify it as less likely than the primary.

Patterns by chief complaint:

Headache → meningitis, subarachnoid hemorrhage, brain tumor, temporal arteritis (age >50), cluster headache

Chest pain → MI, PE, aortic dissection, pneumothorax, pericarditis

Abdominal pain → appendicitis, ectopic pregnancy (women of childbearing age), bowel obstruction, AAA (elderly)

HTN → OSA, secondary HTN (renal, endocrine), pheochromocytoma

Sore throat → GAS pharyngitis, infectious mononucleosis, peritonsillar abscess, epiglottitis (peds)

Pediatric vomiting/diarrhea → severe dehydration, bacterial gastroenteritis vs viral, intussusception, pyloric stenosis (infants)

Pediatric academic decline → vision impairment, hearing impairment, ADHD, learning disability, depression

The rule: When ranking, your must-not-miss goes on the list. It doesn't have to be #1 — but it must be present.

Common DDx Traps

⚠ Selecting too many diagnoses

Picking everything that's even slightly plausible suggests poor clinical judgment. Be selective.

⚠ Missing the must-not-miss

Even if you correctly identify it as less likely, the platform expects it on the list.

⚠ Wrong final-diagnosis ranking

Your top-ranked DDx should match your final diagnosis. Mismatches cost points.

⚠ Using non-platform terminology

iHuman uses fixed diagnosis names. "Severe high blood pressure" doesn't match "Hypertension, Stage 2."

⚠ Not addressing ruled-out conditions

When labs come back normal, explicitly rule out the conditions they ruled out. Silent dismissal loses points.

⚠ Padding the problem statement

Three sentences. Concise. Clinical language. Faculty want diagnostic reasoning, not a re-summary of the entire HPI.

Next steps

Last in the library: Management Plan + SOAP Note — the part that's 100% faculty-scored. Tests to order, the harmful-flag trap, the 6-part management plan structure, and the SOAP note template that faculty look for.

READY FOR THE FULL GUIDE?

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